

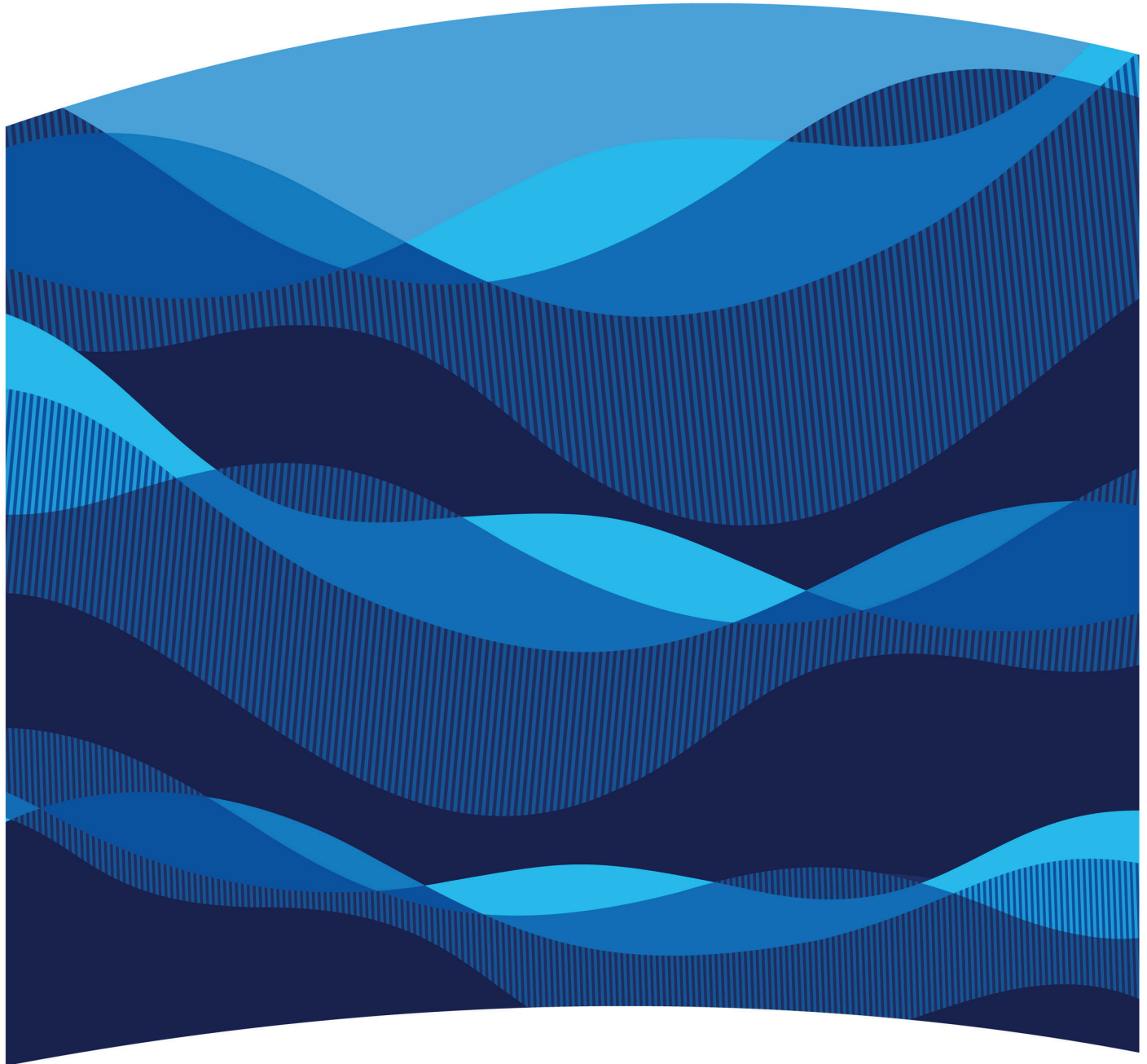
Health Insurance Coverage in the United States: 2022

Current Population Reports

By Katherine Keisler-Starkey, Lisa N. Bunch, and Rachel A. Lindstrom

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Health Insurance Coverage in the United States: 2022

INTRODUCTION

Health insurance offers a means for financing a person's health care expenses. Health insurance coverage provides access to medical care, protection from high unexpected costs, and more economic stability for people and families. While the majority of people in the United States have private health insurance, primarily through an employer, others obtain coverage through programs offered by the government. Yet some do not have health insurance coverage at all (refer to the "What Is Health Insurance Coverage?" text box).

Year-to-year, the rate of health insurance coverage and the distribution of coverage types may change due to economic trends, shifts in the demographic composition of the population such as population aging, and policy changes that affect access to care. Economic changes include returning to work after the COVID-19 pandemic. Policy changes include updates to the Medicaid program in response to the COVID-19 pandemic or other economic or social forces. For example, Congress extended mandated continuous coverage for those with Medicaid, which continued throughout 2022.¹

This report presents statistics on health insurance coverage in the United States in 2022 and changes in health insurance coverage rates between 2021 and 2022. The statistics in this report are based on information collected by the Current Population Survey Annual Social and Economic Supplement (CPS ASEC).*

What Is Health Insurance Coverage?

Health insurance coverage in the Current Population Survey Annual Social and Economic Supplement (CPS ASEC) refers to comprehensive coverage at any time during the calendar year for the civilian, noninstitutionalized population of the United States.* For reporting purposes, the U.S. Census Bureau broadly classifies health insurance coverage as private insurance or public insurance.

Private Coverage

- **Employment-based:** Plan provided through an employer or union.
- **Direct-purchase:** Coverage purchased directly from an insurance company, or through a federal or state Marketplace (e.g., healthcare.gov).
- **TRICARE:** Coverage through TRICARE, formerly known as Civilian Health and Medical Program of the Uniformed Services.

Public Coverage

- **Medicare:** Federal program that helps to pay health care costs for people aged 65 and older and for certain people under the age of 65 with long-term disabilities.
- **Medicaid:** This report uses the term Medicaid to include the specific Medicaid government program and other programs for low-income individuals administered by the states such as Children's Health Insurance Program (CHIP) and Basic Health Programs.
- **VA and CHAMPVA:** Care provided by the Department of Veterans Affairs, the military, and the Civilian Health and Medical Program of the Department of Veterans Affairs.

Additionally, people are considered uninsured if they only had coverage through the Indian Health Service (IHS), as IHS coverage is not considered comprehensive.

* Comprehensive health insurance covers basic health care needs. This definition excludes single service plans such as accident, disability, dental, vision, or prescription medicine plans.

* The U.S. Census Bureau reviewed these data and associated products for unauthorized disclosure of confidential information and approved the disclosure avoidance practices applied to this release: Disclosure Review Board (DRB) approval number: CBDRB-FY23-0432. To further protect respondent privacy, all estimates in this report have undergone additional rounding. As a result, this year's estimates may differ from previous publications and details may not sum to totals. All comparative statements have undergone statistical testing and are statistically significant at the 90 percent confidence level unless otherwise noted.

Small Area Health Insurance Estimates: 2019

Small Area Estimates

Current Population Reports

By Sara Robinson and Katherine Ann Willyard

June 2021

P30-09

INTRODUCTION

This report provides a summary of the 2019 release of the U.S. Census Bureau's Small Area Health Insurance Estimates (SAHIE).¹ SAHIE are the only source of data for single-year estimates of health insurance coverage status for all counties in the United States by selected economic and demographic characteristics (refer to the "Small Area Health Insurance Estimates (SAHIE)" text box).²

The 1-year American Community Survey (ACS) provides detailed estimates of health insurance coverage for counties with populations of 65,000 or more.³ As a data enhancement to the ACS, the SAHIE model-based estimates are a vital source of information for measuring year-to-year change in health insurance coverage at the county level. The data in this report show changes in health insurance coverage between 2018 and 2019, as well as changes in health insurance coverage between 2013 and 2019. In addition, the report provides results on the differences in coverage among selected demographic groups.

¹ The Census Bureau's Disclosure Review Board and Disclosure Avoidance officers reviewed this data product for unauthorized disclosure of confidential information and approved the disclosure avoidance practices applied to this release. CBDRB-FY21-077.

² There are 3,142 counties in the United States. The SAHIE program does not include Kalawao County, Hawaii, due to insufficient data.

³ Approximately 73.6 percent, or 2,313 of U.S. counties, do not have detailed 1-year estimates of health insurance coverage. However, the ACS 1-year county-level estimates cover about 85.0 percent of the total U.S. population. The ACS also releases 1-year supplemental tables of health insurance coverage estimates for geographic areas with populations greater than 20,000, but these tables do not provide the same economic and demographic detail as SAHIE.

HIGHLIGHTS

- From 2018 to 2019, for the population under the age of 65, 90.7 percent of (or 2,850) counties did not have a statistically significant change in their uninsured rate. Among counties that experienced changes in their uninsured rates, more saw an increase (237 counties) than a decrease (54 counties).
- Among the population under the age of 65, the estimated county uninsured rate in 2019 ranged from 2.4 percent to 35.8 percent. The median county uninsured rate was 11.0 percent.
- In 2019, 33.6 percent of (or 1,054) counties had an estimated uninsured rate below 10.0 percent for the population under the age of 65.

OVERVIEW OF SAHIE

Each year, the SAHIE program releases timely, reliable estimates of health insurance coverage for the population under the age of 65 by state and county.⁴ Federal agencies and programs use SAHIE data to determine eligibility for public health services. The SAHIE program is partially funded by the Centers for Disease Control and Prevention's Division of Cancer Prevention and Control (DCPC). The DCPC's National Breast and Cervical Cancer Early Detection Program and its stakeholders use SAHIE to determine the number of low-income, uninsured women who may be

⁴ Please refer to the definition of insured at <www.census.gov/programs-surveys/sahie/about/faq.html>.

eligible for their program at the state and county levels (refer to the “Why Are Small Area Health Insurance Estimates (SAHIE) Important?” text box).

The SAHIE program produces estimates on health insurance coverage at the state and county levels for the full cross-combination of five income-to-poverty ratio (IPR) categories, all incomes, selected age groups, race/ethnicity (state level only), and sex. These IPR categories are defined as the ratio of family income to the federal poverty threshold (refer to the “How Is Poverty Status Measured?” text box). SAHIE data are used to analyze health insurance status by selected characteristics that reflect the federal poverty thresholds and meet the needs of local, state, and federal assistance programs. For instance, the IPR category 0–138 percent of poverty represents the population that may be eligible for Medicaid coverage if they reside in one of the states that expanded Medicaid eligibility under the Patient Protection and Affordable Care Act (ACA).

County-level SAHIE data also allow data users to take a closer look at the distribution and concentration of the uninsured population within states, regions, and metropolitan areas.⁵ Since the SAHIE program produces single-year estimates for all U.S. counties, SAHIE data are used to analyze changes over time in health insurance coverage, as well as geographic variation. The purpose of this report is to

⁵ Reference maps on regions and metropolitan/micropolitan area status are available at <<https://www2.census.gov/programs-surveys/sahie/reference-maps/2019/ref1-mp-2019.pdf>>.

highlight several key findings of such analyses.⁶

HEALTH INSURANCE COVERAGE IN U.S. COUNTIES

In 2019, estimated county uninsured rates for the population under the age of 65 ranged from 2.4 percent to 35.8 percent. The median county uninsured rate was 11.0 percent.⁷ Figure 1 shows how uninsured rates varied among counties throughout the country. The lightest shade in the map represents counties with the lowest uninsured rates (10.0 percent and below). In 2019, 33.6 percent of (or 1,054) counties had an uninsured rate less than 10.0 percent. The Northeast and Midwest had the highest proportion of counties with low uninsured rates.⁸ In 2019, only 18.1 percent of (or 567) counties had uninsured rates greater than 15.0 percent. These counties were primarily located in the South.⁹

⁶ All data shown are estimates containing uncertainty. Sources of uncertainty include model error, sampling error, and nonsampling error. Unless specifically noted in the text, apparent differences among the estimates may not be statistically significant. All direct comparisons cited in the text have been statistically tested at the 90 percent confidence level. More information is available at <www.census.gov/programs-surveys/sahie/technical-documentation/source-and-accuracy.html>.

⁷ The median estimated county uninsured rate differs from the ACS's estimated national uninsured rate, which is 9.2 percent (± 0.1) of the U.S. population under the age of 65 in 2019. The SAHIE program does not produce a national uninsured rate for the United States. SAHIE data are produced using survey estimates from the ACS.

⁸ The number of counties with uninsured rates below 10.0 percent by region: Northeast—188 out of 217 counties (86.6 percent); Midwest—554 out of 1,055 counties (52.5 percent); South—213 out of 1,422 counties (15.0 percent); West—99 out of 447 counties (22.2 percent).

⁹ Among the 567 counties with uninsured rates above 15.0 percent, 82.5 percent (468 counties) are in the South. The remaining are in the Midwest (57 counties) and West (42 counties). No counties in the Northeast fell into this category.

SMALL AREA HEALTH INSURANCE ESTIMATES (SAHIE)

SAHIE are model-based enhancements of the American Community Survey (ACS) estimates, created by integrating additional information from administrative records, postcensal population estimates, and decennial census data. SAHIE methodology employs statistical modeling techniques to combine this supplemental information with survey data to produce estimates that are more reliable. SAHIE are broadly consistent with the direct ACS survey estimates, but with help from other data sources, SAHIE program estimates are more precise than the ACS 1-year and 5-year survey estimates for most counties. Detailed ACS 1-year estimates are not available for most of these smaller geographic areas. A 2019 ACS map of unpublished counties is available at <<https://www2.census.gov/programs-surveys/sahie/reference-maps/2019/ref2-mp-2019.pdf>>.

Information on the various input data sources used in producing SAHIE is available at <www.census.gov/programs-surveys/sahie/technical-documentation/model-input-data.html>.

SAHIE are subject to several types of uncertainty. Details on this and the SAHIE methodology are available at <www.census.gov/programs-surveys/sahie/technical-documentation/methodology.html>.

Health Insurance Coverage: Early Release of Estimates From the National Health Interview Survey, 2023

by Robin A. Cohen, Ph.D., Elizabeth M. Briones, Ph.D., and Michael E. Martinez, M.P.H., M.H.S.A.
Division of Health Interview Statistics, National Center for Health Statistics

What's New

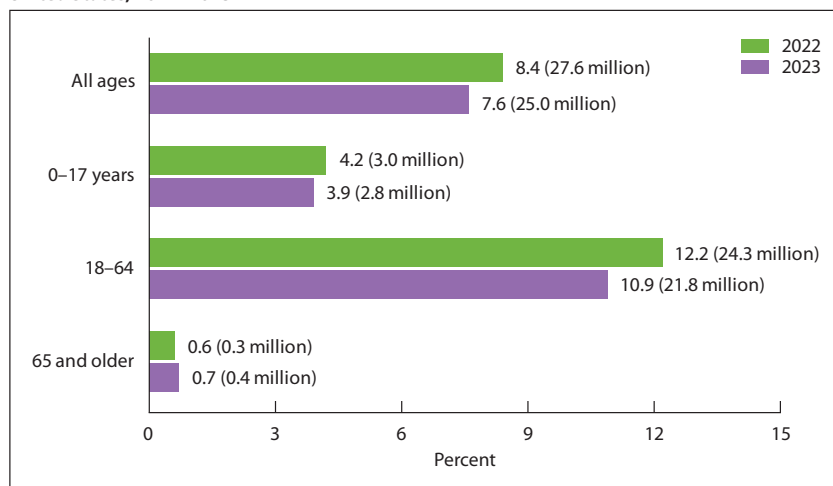
- This report presents health insurance coverage estimates from the 2023 National Health Interview Survey, along with selected trends from 2019 through 2023.

Highlights

- In 2023, 25.0 million people of all ages (7.6%) were uninsured at the time of interview. This was lower than, but not significantly different from 2022, when 27.6 million people of all ages (8.4%) were uninsured.
- In 2023, among adults ages 18–64, 10.9% were uninsured at the time of interview, 23.0% had public coverage, and 68.1% had private health insurance coverage.
- The percentage of adults ages 18–64 who were uninsured in 2023 (10.9%) was lower than the percentage who were uninsured in 2022 (12.2%).
- Among children ages 0–17 years, 3.9% were uninsured, 44.2% had public coverage, and 54.0% had private health insurance coverage.
- The percentage of people younger than age 65 with exchange-based coverage increased from 3.7% in 2019 to 4.8% in 2023.

This report presents estimates of health insurance coverage for the U.S. civilian noninstitutionalized population based on data from the January–December 2023 National Health Interview Survey (NHIS). The National Center for Health Statistics is releasing these estimates before final editing and final weighting to provide access to the most recent information from NHIS. Estimates are disaggregated by age group, sex, family income (as a percentage of the federal poverty level [FPL]), race and ethnicity, and state Medicaid expansion status. Detailed appendix tables contain all estimates presented in the figures and additional estimates from NHIS for selected population characteristics. With 5 years of comparable data available starting with the redesigned NHIS in 2019, this report provides data on trends, similar to reports using 2018 data and earlier. Quarterly estimates by age group and family income, and more information about NHIS and the Early Release Program, are available from the [NHIS website](#).

Figure 1. Percentage and number of people who were uninsured, by age group and year: United States, 2022–2023

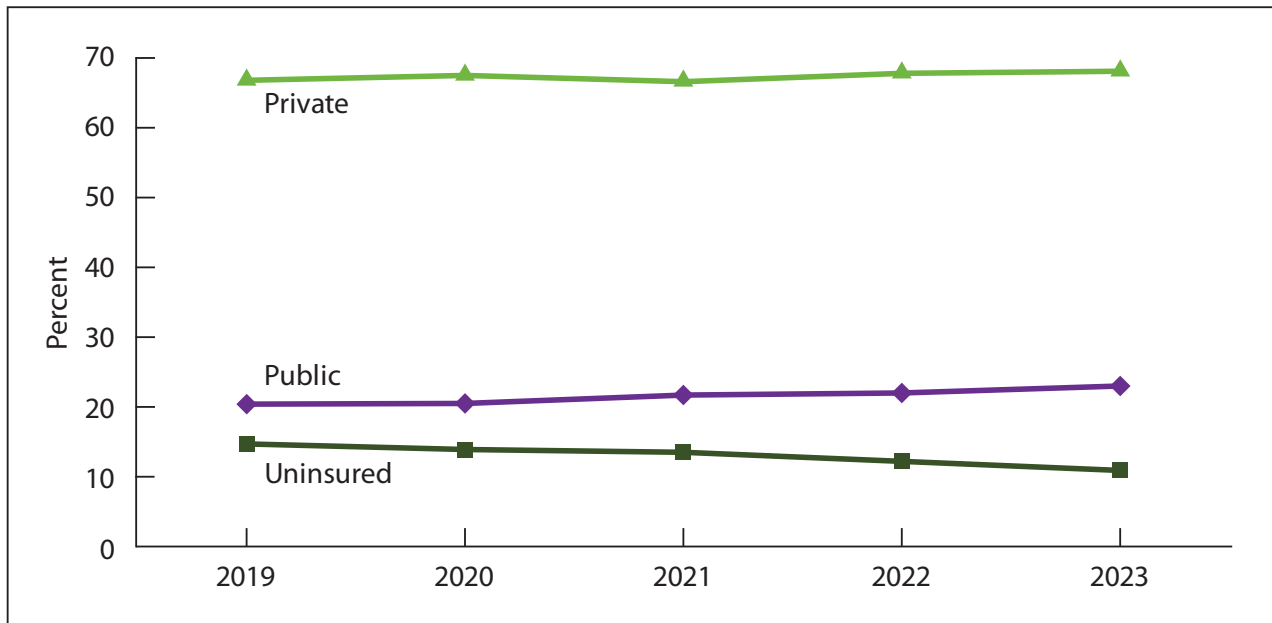


NOTES: People were defined as uninsured if they did not have any private health insurance, Medicare, Medicaid, Children's Health Insurance Program (CHIP), state-sponsored or other government plan or military plan. People were also defined as uninsured if they had only Indian Health Service coverage or had only a private plan that paid for one type of service, such as accidents or dental care. Data are based on household interviews of a sample of the U.S. civilian noninstitutionalized population.
SOURCE: National Center for Health Statistics, National Health Interview Survey, 2022–2023.

- In 2023, 25.0 million people of all ages (7.6%) were uninsured at the time of interview. This was lower than, but not significantly different from 2022, when 27.6 million people of all ages (8.4%) were uninsured (Figure 1).
- In 2023, 21.8 million adults ages 18–64 (10.9%) were uninsured. This was lower than 2022, when 24.3 million adults ages 18–64 (12.2%) were uninsured.

- In 2023, no significant changes were observed in the number and percentage of uninsured for children ages 0–17 years or adults age 65 and older.
- In 2023, adults ages 18–64 were the most likely to be uninsured (10.9%), followed by children ages 0–17 years (3.9%) and adults age 65 and older (0.7%).

Figure 2. Percentage of adults ages 18–64 who were uninsured or had public or private coverage, by year: United States, 2019–2023



NOTES: People were defined as uninsured if they did not have any private health insurance, Medicare, Medicaid, Children's Health Insurance Program (CHIP), state-sponsored or other government plan or military plan. People were also defined as uninsured if they had only Indian Health Service coverage or had only a private plan that paid for one type of service, such as accidents or dental care. Public coverage includes Medicaid, CHIP, state-sponsored or other government-sponsored health plan, Medicare, and military plans. Private coverage includes any comprehensive private insurance plan (including health maintenance and preferred provider organizations). These plans include those obtained through an employer, purchased directly, purchased through local or community programs, or purchased through the Health Insurance Marketplace or a state-based exchange. Private coverage excludes plans that pay for only one type of service, such as accidents or dental care. A small number of people were covered by both public and private plans and were included in both categories. Data are based on household interviews of a sample of the U.S. civilian noninstitutionalized population.

SOURCE: National Center for Health Statistics, National Health Interview Survey, 2019–2023.

- The percentage of adults ages 18–64 who were uninsured decreased from 14.7% in 2019 to 10.9% in 2023. Public coverage increased from 2019 (20.4%) through 2023 (23.0%). Private coverage increased from 2019 (66.8%) through 2023 (68.1%) (Figure 2).
- Among adults ages 18–64, the percentage who were uninsured decreased between 2022 (12.2%) and 2023 (10.9%).
- The percentage of adults who had public coverage in 2023 (23.0%) was higher but not significantly different from the percentage who had public coverage in 2022 (22.0%).
- The percentage of adults who had private coverage in 2023 (68.1%) was not significantly different from the percentage who had private coverage in 2022 (67.8%).

National Health Statistics Reports

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Demographic Variation in Health Insurance Coverage: United States, 2022

by Robin A. Cohen, Ph.D., and Emily P. Terlizzi, M.P.H.

Abstract

Objectives—This report presents national estimates of different types of health insurance coverage and lack of coverage (uninsured). Estimates are presented by selected sociodemographic characteristics, including age, sex, race and Hispanic origin, family income, education level, employment status, and marital status.

Methods—Data from the 2022 National Health Interview Survey were used to estimate health insurance coverage. Estimates were categorized by selected sociodemographic characteristics. Additionally, those who were uninsured were categorized by length of time since they had coverage, private coverage was further classified by source of plan, and public coverage was categorized by type of public plan.

Results—In 2022, 28.1 million (8.6%) people of all ages were uninsured at the time of the interview. This includes 27.7 million (10.2%) people younger than age 65. Among children, 3.0 million (4.2%) were uninsured, and among working-age adults (ages 18–64), 24.7 million (12.4%) were uninsured. Among people younger than age 65, 64.0% were covered by private health insurance, including 56.0% with employment-based coverage and 6.8% with directly purchased coverage. Moreover, 4.5% were covered by exchange-based coverage, a type of directly purchased coverage. Among people younger than age 65, about two in five children and one in five adults ages 18–64 had public health coverage, mainly Medicaid and the Children's Health Insurance Program. Among adults age 65 and older, the percentage who were covered by private health insurance (with or without Medicare), Medicare Advantage, and traditional Medicare only varied by age, family income, education level, and race and Hispanic origin.

Keywords: uninsured • private health insurance • public coverage • National Health Interview Survey

health, and mitigate healthcare disparities (4). Although marked decreases in the percentage of uninsured people post-ACA occurred, racial and ethnic disparities in health insurance coverage persist (5,6). Also, previous research has noted differences in health insurance coverage by age, education level, employment status, marital status, and poverty level (7,8). Population estimates of health insurance coverage are essential to develop and assess federal and state health care coverage programs and policies (9).

This report provides the most current description of health insurance coverage by demographic characteristics annually (10). Estimates of the percentage and number of people who were uninsured, had private coverage, and had public coverage at the time of the interview are presented. Estimates are also categorized by duration (length of time) of being uninsured, source of private coverage, and type of public coverage. For adults age 65 and older, six mutually exclusive categories of coverage are presented by selected sociodemographic characteristics.

Introduction

Health insurance coverage in the United States is linked to improved health care and health outcomes and

is a key measure of health care access (1,2). The passage of the Affordable Care Act (ACA) in 2010 (3) was designed to increase access to health care, improve



U.S. DEPARTMENT OF HEALTH AND HUMAN SERVICES
Centers for Disease Control and Prevention
National Center for Health Statistics



NCHS reports can be downloaded from: <https://www.cdc.gov/nchs/products/index.htm>.

Methods

Data source

Estimates in this report are based on data from the Sample Adult and Sample Child modules of the 2022 National Health Interview Survey (NHIS). NHIS is a nationally representative household survey of the U.S. civilian noninstitutionalized population. It is conducted continuously throughout the year by the National Center for Health Statistics. From each household, one sample adult is randomly selected to answer detailed questions about their health. One sample child, if present, is also randomly selected from each household, and an adult who is knowledgeable and responsible for the child's health answers questions on the child's behalf. Interviews are typically conducted in respondents' homes, but follow-ups to complete interviews may be conducted over the telephone when necessary. In 2022, 55.7% of the Sample Adult interviews and 56.1% of the Sample Child interviews were conducted at least partially by telephone (11). The 2022 NHIS Sample Adult and Sample Child response rates were 47.7% and 45.8%, respectively (11).

Both the Sample Adult and Sample Child modules include a full range of questions addressing health insurance, such as coverage status, sources of coverage, characteristics of coverage, and reasons for no coverage. The sample adult and sample child receive a similar set of health insurance questions, so the Sample Adult and Sample Child files can be combined to create a file that contains people of all ages. Estimates are based on a combined file containing 35,115 people (7,464 sample children and 27,651 sample adults).

In this report, “adults” refers to people age 18 and older, and “children” refers to people younger than age 18 years. “Working-age adult” refers to people ages 18–64, and “older adult” refers to people age 65 and older.

Insurance coverage

People were considered uninsured if, at the time of the interview, they did not have coverage through private health insurance, Medicare, Medicaid, Children's Health Insurance Program (CHIP), military (TRICARE, Veterans Administration [VA], and CHAMP–VA), other state-sponsored health plans, or other government programs. People also were defined as uninsured if they only had Indian Health Service (IHS) coverage or only had a private plan that paid for one type of service, such as dental, vision, or prescription drugs. Uninsured people were further classified into one of three categories according to the length of time since they last had coverage (uninsured for less than a year, uninsured for a year or more, and uninsured for unknown duration). Length of time since a person last had coverage was based on the following question: “How long has it been since [you/child's name] last had health care coverage that paid for doctor's visits or hospital stays?”

Private health insurance coverage includes any comprehensive private insurance plan (including health maintenance and preferred provider organizations). These plans include those obtained through an employer, purchased directly, purchased through local or community programs, or purchased through the Health Insurance Marketplace or a state-based exchange. Private coverage excludes plans that pay for only one type of service, such as dental, vision, or prescription drugs. People with private coverage were further classified into three sources of private coverage: employment-based, directly purchased, and other sources. The employment-based category includes plans obtained through an employer, union, or other professional organization. Directly purchased coverage includes exchange-based coverage in addition to plans obtained directly from an insurance company or through a broker. Exchange-based coverage is a private plan purchased through the federal Health Insurance Marketplace or state-based exchanges that were established as part of ACA (3). The “other sources” category for private health insurance includes plans obtained through state or local

government or community programs, school, parent, other relative, other source not specified, and those who did not respond to the question asking about the source of their private coverage (refused, not ascertained, and don't know).

Public health plan coverage includes Medicaid, CHIP, state-sponsored or other government-sponsored health plans, Medicare, and military plans. For people younger than age 65, public coverage was classified into four categories: Medicaid and CHIP, Medicare, other government, and military coverage. The Medicaid and CHIP category also includes those with state-sponsored plans. These categories are not mutually exclusive, and a person may be covered by more than one type of public coverage. Therefore, a person may be included in multiple categories.

For adults age 65 and older, a health insurance hierarchy of six mutually exclusive categories was developed. This hierarchy eliminates duplicate responses for both private health insurance and Medicare Advantage, giving preference to the report of Medicare Advantage. Medicare Advantage is another way for people covered by Medicare to get their Medicare Part A and Medicare Part B coverage. Medicare Advantage plans are sometimes called “Part C” and are offered by Medicare-approved companies that must follow rules set by Medicare (12). Older adults with more than one type of health insurance were assigned to the first appropriate category in the following hierarchy:

Private coverage—Includes older adults who have both Medicare and any comprehensive private health insurance plan (including health maintenance organizations, preferred provider organizations, and Medigap plans). This category also includes older adults with private insurance only but excludes those with a Medicare Advantage plan.

Medicare and Medicaid (dual-eligible)—Includes older adults who do not have any private coverage but have Medicare and Medicaid or other state-sponsored health plans, including CHIP.

Medicare Advantage—Includes older adults who only have Medicare coverage through a Medicare Advantage plan.

Traditional Medicare only (excluding Medicare Advantage)—Includes older adults who only have Medicare coverage

Covered by Some Type of Health Insurance

All Persons		Under 19 Years		Under 65 Years	
State	Percent	State	Percent	State	Percent
Massachusetts	97.6 (0.2)	District of Columbia	98.6 (0.9)	Massachusetts	97.2 (0.2)
District of Columbia	97.1 (0.5)	Massachusetts	98.5 (0.3)	District of Columbia	96.7 (0.6)
Hawaii	96.5 (0.4)	Rhode Island	97.9 (0.7)	Hawaii	95.7 (0.5)
Vermont	96.1 (0.5)	New York	97.4 (0.2)	Vermont	95.1 (0.6)
Rhode Island	95.8 (0.6)	Vermont	97.4 (0.9)	Rhode Island	94.9 (0.7)
Iowa	95.5 (0.2)	Michigan	97.3 (0.2)	Minnesota	94.7 (0.2)
Michigan	95.5 (0.2)	Washington	97.2 (0.3)	Iowa	94.6 (0.3)
Minnesota	95.5 (0.2)	West Virginia	97.2 (0.7)	Michigan	94.5 (0.2)
New Hampshire	95.1 (0.4)	Hawaii	97 (0.6)	New York	94.3 (0.1)
New York	95.1 (0.1)	Oregon	97 (0.4)	New Hampshire	94 (0.5)
Connecticut	94.8 (0.3)	Alabama	96.9 (0.4)	Connecticut	93.9 (0.4)
Wisconsin	94.8 (0.2)	California	96.8 (0.2)	Wisconsin	93.7 (0.2)
Pennsylvania	94.7 (0.2)	Connecticut	96.8 (0.5)	Pennsylvania	93.5 (0.2)
Delaware	94.4 (0.6)	Minnesota	96.7 (0.3)	Kentucky	93.3 (0.4)
Kentucky	94.4 (0.3)	Delaware	96.6 (0.9)	Delaware	93.1 (0.8)
Ohio	94.1 (0.2)	Illinois	96.6 (0.3)	Maryland	92.9 (0.3)
West Virginia	94.1 (0.4)	New Hampshire	96.6 (0.7)	Ohio	92.9 (0.2)
Oregon	94 (0.3)	Louisiana	96.5 (0.4)	Washington	92.8 (0.2)
Maryland	93.9 (0.3)	Iowa	96.3 (0.4)	Oregon	92.7 (0.4)
Washington	93.9 (0.2)	New Mexico	96.2 (0.7)	West Virginia	92.6 (0.5)
North Dakota	93.6 (0.7)	New Jersey	96.1 (0.3)	California	92.5 (0.1)
California	93.5 (0.1)	Maryland	95.8 (0.5)	North Dakota	92.5 (0.8)
Virginia	93.5 (0.2)	Kentucky	95.7 (0.6)	Virginia	92.4 (0.2)
Illinois	93.4 (0.2)	Virginia	95.6 (0.4)	Illinois	92.3 (0.2)
Maine	93.4 (0.5)	Ohio	95.5 (0.3)	Nebraska	92.2 (0.5)
Nebraska	93.3 (0.4)	Wisconsin	95.5 (0.4)	New Jersey	92 (0.3)
New Jersey	93.2 (0.2)	Maine	95.4 (1)	Colorado	91.7 (0.3)
Louisiana	93.1 (0.3)	Nebraska	95.4 (0.7)	Indiana	91.7 (0.3)
Indiana	93 (0.2)	South Carolina	95.3 (0.5)	Louisiana	91.7 (0.3)
Colorado	92.9 (0.3)	Colorado	95.2 (0.4)	Maine	91.6 (0.6)
United States	92 (0.1)	North Carolina	95.2 (0.3)	Utah	90.9 (0.5)
South Dakota	91.9 (0.5)	United States	94.9 (0.1)	United States	90.5 (0.1)
Utah	91.9 (0.5)	Kansas	94.8 (0.6)	Idaho	90.3 (0.6)
Idaho	91.8 (0.5)	Pennsylvania	94.8 (0.3)	South Dakota	90.2 (0.7)
New Mexico	91.8 (0.5)	Tennessee	94.7 (0.5)	New Mexico	90 (0.6)
Montana	91.7 (0.6)	Indiana	94.5 (0.4)	Arkansas	89.9 (0.5)
Arkansas	91.6 (0.4)	Mississippi	94.5 (0.6)	Montana	89.8 (0.7)
Kansas	91.4 (0.3)	North Dakota	94.5 (1.1)	Kansas	89.7 (0.4)
Missouri	91.4 (0.3)	Idaho	94.3 (0.8)	Missouri	89.7 (0.3)
Alabama	91.2 (0.3)	Missouri	94.2 (0.4)	Alabama	89.4 (0.4)
South Carolina	90.9 (0.3)	Arkansas	94.1 (0.8)	North Carolina	88.9 (0.3)
North Carolina	90.7 (0.2)	Utah	93.9 (0.8)	South Carolina	88.9 (0.4)
Tennessee	90.7 (0.3)	Georgia	93.8 (0.4)	Tennessee	88.9 (0.3)
Arizona	89.7 (0.3)	South Dakota	93.8 (0.9)	Arizona	87.5 (0.4)
Mississippi	89.2 (0.4)	Montana	93.1 (1.3)	Alaska	87.4 (0.9)
Alaska	89 (0.8)	Oklahoma	92.7 (0.4)	Mississippi	86.9 (0.5)
Nevada	88.9 (0.4)	Florida	92.6 (0.4)	Nevada	86.8 (0.5)
Florida	88.8 (0.2)	Nevada	92.4 (0.8)	Georgia	86.4 (0.3)
Wyoming	88.5 (1.2)	Wyoming	92.1 (1.7)	Oklahoma	86.2 (0.3)
Georgia	88.3 (0.2)	Alaska	91.6 (1.3)	Florida	86.1 (0.2)
Oklahoma	88.3 (0.3)	Arizona	91.6 (0.6)	Wyoming	86 (1.4)
Texas	83.4 (0.2)	Texas	89.1 (0.4)	Texas	81.1 (0.2)

Note: Numbers in thousands; Figures cover civilian noninstitutionalized population in 2022; N/A indicates that data was not available; Z represents or rounds to zero; Margin of error appears in parenthesis and is calculated using replicate weights.

Source: U.S. Census Bureau, American Community Survey, Health Insurance Coverage Status and Type of Coverage by State and Age for All People: 2022

Covered by Private Health Insurance

All Persons		Under 19 Years		Under 65 Years	
State	Percent	State	Percent	State	Percent
North Dakota	78.4 (1.1)	Utah	79.7 (1.2)	Utah	81.5 (0.7)
Utah	78.4 (0.6)	North Dakota	76.3 (2.3)	North Dakota	80.2 (1.2)
New Hampshire	75.6 (0.8)	New Hampshire	72.7 (2.1)	New Hampshire	79 (0.9)
Hawaii	75 (0.8)	Wyoming	71.6 (3.1)	Nebraska	76.9 (0.8)
Minnesota	74.7 (0.5)	Minnesota	70.3 (1)	South Dakota	76.9 (1.1)
Kansas	73.7 (0.6)	Massachusetts	70 (0.9)	Wisconsin	76.8 (0.5)
Nebraska	73.6 (0.7)	Wisconsin	70 (0.8)	Hawaii	76.7 (1)
Massachusetts	73.4 (0.4)	Nebraska	69.8 (1.5)	Kansas	76.4 (0.6)
Iowa	73.3 (0.5)	Rhode Island	69.4 (2.8)	Minnesota	76.2 (0.5)
South Dakota	73.3 (0.9)	South Dakota	69.2 (1.9)	Iowa	76 (0.6)
Wisconsin	73.3 (0.4)	Kansas	68.9 (1.3)	Massachusetts	76 (0.5)
Virginia	73 (0.4)	Virginia	68.4 (0.8)	Virginia	75.2 (0.4)
Maryland	72.8 (0.5)	Hawaii	67.3 (2)	District of Columbia	74.2 (1.7)
District of Columbia	72.5 (1.5)	Iowa	67.3 (1.2)	New Jersey	74.2 (0.4)
Delaware	71.8 (1.2)	New Jersey	66.4 (0.8)	Rhode Island	74.2 (1.5)
New Jersey	71.2 (0.4)	Maine	66.1 (2.1)	Wyoming	74.2 (1.9)
Pennsylvania	71.2 (0.3)	Colorado	66 (1.3)	Colorado	73.8 (0.5)
Wyoming	71 (1.6)	Connecticut	65.3 (1.4)	Maryland	73.7 (0.6)
Washington	70.8 (0.4)	Delaware	64.5 (2.9)	Pennsylvania	73.7 (0.3)
Colorado	70.5 (0.5)	Michigan	64.5 (0.8)	Washington	73.3 (0.5)
Rhode Island	70.5 (1.3)	Idaho	64.4 (1.7)	Delaware	73 (1.4)
Michigan	70.4 (0.4)	Maryland	64.2 (1.2)	Missouri	72.6 (0.5)
Vermont	70 (1.2)	Pennsylvania	64.2 (0.7)	Connecticut	72.5 (0.7)
Illinois	69.3 (0.4)	Washington	64.1 (0.9)	Maine	72.2 (1.1)
Connecticut	68.9 (0.6)	Ohio	64 (0.7)	Vermont	72.1 (1.4)
Missouri	68.8 (0.5)	Oregon	63.7 (1.6)	Illinois	72 (0.5)
Maine	68.7 (1)	Illinois	63.3 (0.8)	Michigan	71.4 (0.4)
Indiana	68.4 (0.5)	Missouri	63.2 (0.9)	Ohio	71.1 (0.4)
Idaho	68.3 (0.9)	Indiana	62.5 (1.1)	Indiana	70.9 (0.6)
Ohio	68.1 (0.4)	Vermont	62.5 (2.9)	Idaho	70.7 (1)
North Carolina	67.8 (0.4)	Alaska	60.7 (2.3)	North Carolina	70.4 (0.4)
Tennessee	67.2 (0.5)	United States	60.6 (0.2)	Oregon	70.1 (0.7)
United States	67.2 (0.1)	Nevada	59.8 (1.8)	Tennessee	70 (0.6)
Oregon	67.1 (0.6)	Tennessee	59.8 (1)	United States	69.8 (0.2)
Alabama	67 (0.5)	New York	59.6 (0.7)	South Carolina	69.1 (0.6)
South Carolina	66.9 (0.5)	District of Columbia	59.3 (4.2)	Georgia	69 (0.5)
Georgia	66.6 (0.4)	Arizona	59 (1.1)	Alabama	68.9 (0.6)
Alaska	66.4 (1.2)	California	58.6 (0.4)	Florida	68.1 (0.4)
New York	65.4 (0.3)	Montana	58.5 (2.2)	New York	67.7 (0.4)
Montana	65.2 (1)	North Carolina	58.2 (0.8)	Montana	67.6 (1.3)
Arizona	63.9 (0.5)	Georgia	57.2 (0.9)	Alaska	66.8 (1.3)
California	63.8 (0.2)	South Carolina	56.1 (1.3)	Arizona	66.8 (0.6)
Florida	63.6 (0.3)	Alabama	55.4 (0.9)	California	66.7 (0.2)
Nevada	63.4 (0.8)	Florida	55.3 (0.7)	Nevada	66.7 (0.9)
Kentucky	62.8 (0.6)	Texas	54.3 (0.6)	Texas	64.9 (0.3)
Texas	62.8 (0.3)	Kentucky	53.8 (1)	Kentucky	64 (0.7)
West Virginia	61.8 (1)	West Virginia	52.5 (2.1)	Mississippi	63.7 (0.7)
Oklahoma	61.7 (0.5)	Oklahoma	51 (0.9)	Oklahoma	62.8 (0.6)
Mississippi	61.5 (0.6)	Mississippi	50 (1.4)	West Virginia	62.2 (1.3)
Arkansas	60 (0.8)	Louisiana	49.3 (1.4)	Arkansas	61.4 (0.9)
Louisiana	58.1 (0.7)	Arkansas	47.9 (1.5)	Louisiana	59.4 (0.8)
New Mexico	54.4 (1)	New Mexico	44.5 (2)	New Mexico	55.4 (1.2)

Note: Numbers in thousands; Figures cover civilian noninstitutionalized population in 2022; N/A indicates that data was not available; Z represents or rounds to zero; Margin of error appears in parenthesis and is calculated using replicate weights.

Source: U.S. Census Bureau, American Community Survey, Health Insurance Coverage Status and Type of Coverage by State and Age for All People: 2022

GUIDE TO U.S. HMOs & PPOs

ALABAMA

Health Insurance Coverage Status and Type of Coverage by Age

Category	All Persons		Under 19 years		Under 65 years	
	Number	%	Number	%	Number	%
Total population	4,989 (2)	100 (0)	1,183 (6)	100 (0)	4,099 (4)	100 (0)
Covered by some type of health insurance	4,551 (17)	91.2 (0.3)	1,145 (8)	96.9 (0.4)	3,665 (17)	89.4 (0.4)
Covered by private health insurance	3,342 (25)	67 (0.5)	655 (12)	55.4 (0.9)	2,824 (23)	68.9 (0.6)
Employer-based	2,675 (26)	53.6 (0.5)	553 (12)	46.7 (1)	2,375 (26)	57.9 (0.6)
Direct purchase	714 (16)	14.3 (0.3)	91 (7)	7.7 (0.5)	476 (15)	11.6 (0.4)
TRICARE	245 (10)	4.9 (0.2)	45 (5)	3.8 (0.4)	147 (8)	3.6 (0.2)
Covered by public health insurance	1,917 (20)	38.4 (0.4)	548 (14)	46.4 (1.1)	1,052 (21)	25.7 (0.5)
Medicaid	1,023 (21)	20.5 (0.4)	543 (13)	45.9 (1.1)	893 (20)	21.8 (0.5)
Medicare	1,038 (8)	20.8 (0.2)	8 (2)	0.7 (0.2)	174 (8)	4.2 (0.2)
VA Care	142 (7)	2.8 (0.1)	1 (1)	0.1 (Z)	74 (6)	1.8 (0.1)
Not covered at any time during the year	437 (17)	8.8 (0.3)	37 (5)	3.1 (0.4)	434 (17)	10.6 (0.4)

Note: Numbers in thousands; Figures cover civilian noninstitutionalized population in 2022; N/A indicates that data was not available; Z represents or rounds to zero; Margin of error appears in parenthesis and is calculated using replicate weights.

Source: U.S. Census Bureau, American Community Survey, Health Insurance Coverage Status and Type of Coverage by State and Age for All People: 2022

Alabama

1 Aetna Health of Alabama

151 Farmington Avenue

Hartford, CT 06156

Toll-Free: 800-872-3862

www.aetna.com

Subsidiary of: CVS Health / Aetna Inc.

For Profit Organization: Yes

Healthplan and Services Defined

PLAN TYPE: PPO

Model Type: Network

Plan Specialty: Behavioral Health, Lab, PBM, Radiology

Benefits Offered: Behavioral Health, Dental, Disease

Management, Long-Term Care, Physical Therapy, Podiatry,

Prescription, Psychiatric, Vision, Wellness, Life, LTD, STD

Type of Coverage

Commercial, Supplemental Medicare, Student health

Type of Payment Plans Offered

POS, FFS

Geographic Areas Served

Statewide

Key Personnel

Market Pres., GA & Gulf. Cynthia Follmer

2 Ascension At Home

1900 International Park Dr.

Suite 250

Birmingham, AL 35243

Toll-Free: 833-380-9583

Phone: 205-313-2800

ascensionathome.com

Subsidiary of: Ascension, Compassus

Non-Profit Organization: Yes

Year Founded: 2014

Healthplan and Services Defined

PLAN TYPE: Other

Plan Specialty: Disease Management

Benefits Offered: Dental, Disease Management, Home Care,

Wellness, Ambulance & Transportation; Nursing Service;

Short-and-long-term care management planning; Hospice

Geographic Areas Served

Texas, Alabama, Indiana, Kansas, Michigan, Mississippi,

Oklahoma, Wisconsin

Key Personnel

President, Ascension Eduardo Conrado

CEO, Compassus David Grams

3 Behavioral Health Systems

2 Metroplex Drive

Suite 500

Birmingham, AL 35209

Toll-Free: 800-245-1150

www.behavioralhealthsystems.com

For Profit Organization: Yes

Year Founded: 1989

Total Enrollment: 502,000

Healthplan and Services Defined

PLAN TYPE: PPO

Plan Specialty: Behavioral Health

Benefits Offered: Behavioral Health, Prescription,

Psychiatric, Wellness, Worker's Compensation, EAP, Drug

Testing

Geographic Areas Served

Nationwide

Network Qualifications

Minimum Years of Practice: 5

Pre-Admission Certification: Yes

Peer Review Type

Utilization Review: Yes

Case Management: Yes

Publishes and Distributes Report Card: Yes

Accreditation Certification

AAAH, TJC, URAC, CARF

Utilization Review, Pre-Admission Certification, Quality

Assurance Program

Key Personnel

Founder/Chairman/CEO. Deborah L. Stephens

President, Safety First Danny Cooner

EVP/Chief Clinical Off. Kyle Strange

SVP/Chief Financial Off. Tina Cannon

Medical Director. William Patterson, MD

EVP/Chief Marketing Off. Shannon Flanagan

Corp. VP, Business Dev. Jeff Hill

Public & Corp. Relations Shannon Flanagan

205-443-5483

Specialty Managed Care Partners

State of Alabama, Drummond Co, MTD Products

Enters into Contracts with Regional Business Coalitions: Yes

Employers Coalition on Healthcare Options (ECHO),

Louisiana Business Group on Health (LBGH), Louisiana

Health Care Alliance (LHCA)

4 Blue Cross and Blue Shield of Alabama

450 Riverchase Parkway East

Birmingham, AL 35244

Toll-Free: 888-267-2955

www.bcbsal.org

Year Founded: 1936

State Enrollment: 2,900,000

Healthplan and Services Defined

PLAN TYPE: HMO/PPO

Model Type: IPA

Plan Specialty: Behavioral Health, Dental, Lab

Benefits Offered: Behavioral Health, Dental, Physical

Therapy, Prescription, Wellness

Type of Coverage

Commercial, Individual, Medicare, Supplemental Medicare

Geographic Areas Served

Statewide